

--

Name: نادية ربيع عبدالقادر محمد National ID: 26703190102181	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية العراق
وزارة الدفاع
القوة الجوية

الجنرال
عليه السلام
عليه السلام

القوة الجوية
القوة الجوية

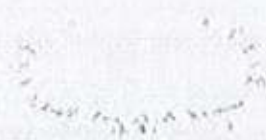
٢٠١٧-٠٣-١٩-٠١-٠٢١١ ٨١

JG2830639



٢٠٢٢/٠٣-٢٠١٧-٠٣-١٩-٠١-٠٢١١ ٨١
الجنرال
عليه السلام
عليه السلام





Ministry of Health
Kingdom of Saudi Arabia

الوزارة
الصحة العامة



MDT form

A-Patient ID:

1- Name: تاليا دويح عبد الله

Sex:

3- Medical numbers:

Female

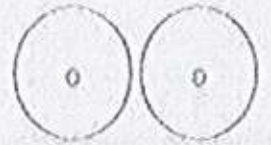
83042923

Old Mdt:

3750/2021

2- ID number: 25703190102181

Patient Age: 55 Y



Family history

Phone:

01114454795

BMI

REASON OF MDT

REMDT

marital status

NRS Score

GROUP

H

Date of MDT

2023-05-14

Comorbidity

CARDIOMYOPATHY

Metabolic status

Clinical data

Breast Imaging:

Mets Workup:

DX:

ER:

7/8

PR:

8/8

Her 2 Neu:

3

SISH:

Ki67:

40

Y

N

M

CY 3

1

0

Sonomammography

PATIENT HISTORY

T3N1M0-->NACT YCHPX18 WK WITH COMPLICATED CARDIOMYOPATHY-->BCS+LND 4/2021
 STARTED PORTH--> ON ADJ ARAMIDEX+TDM,, FINISHED 12 CYCLE LAST ONE ON 2/4/2023
 NEW CTCAP-->REQUESTED+ NEW DOPPLER REQUESTED
 CTCAP 1/2023-->NO METS
 HISTORY OF DVT IN IJV--> ON RIVAROSPIRE AFTER 3 MONTHS OF HEPARIN

MDT OUTCOME & PLAN

US A/P ONLY FAT NECROSIS TO CONTINUE AT

SIGNATURE

MEDICAL ONCOLOGY

RADIOLOGY

SURGERY

PATHOLOGY

Psychiatric

Nutrition

Name: <u>ناديه ربيع عبدالقادر</u>		
MR. : <u>2934</u>		
Detailed Medical Report		
		السيد
تخية طية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر ف بأن نوضح فيما يلي تقريرنا عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient's tumor biology was documented as ER 7/8, PR 8/8, HER2 negative, Neu 3, SISH negative, and Ki-67 40 %. Staging was recorded as T3 N1 M0 based on CT imaging. The clinical history includes neoadjuvant chemotherapy (NACT) with a TCHP regimen for 18 weeks, complicated by cardiomyopathy, followed by breast-conserving surgery with lymph node dissection performed in April 2021. Post-operative therapy consisted of adjuvant aramidex plus TDM, which was completed after 12 cycles, the last dose administered on 2 April 2023. Subsequent work-up requested a new CT CAP and Doppler study; a CT CAP performed in January 2023 showed no metastases. The patient has a prior history of internal jugular vein thrombosis and is currently on rivaroxaban after three months of heparin therapy. Ongoing management includes continuation of femara, calcium 1000 mg daily (1 tablet × 1 × 90), and a single dose of alpha-interferon for soft-tissue sarcoma. The patient has completed 12 cycles of aramidex plus TDM.		
تحريرا في ,		
	الوقت :	التاريخ : 20-09-2026
	الطبيب المعالج : <u>ايه مدحت بدوي محمد</u>	
يعتمد ,		



جهة الإصدار: طرح الجيزة

تاريخ الإصدار: 2023-1-23

التي من يهيمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للتأمن الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "المقوى العام والمعادلات والأعمال" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 "الأطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة العاملة .
وطبقا لقاعدة البراهات المرسلة من التأمينات الاجتماعية
وطبقا لقاعدة البراهات المرسلة من التأمين الاجتماعي
والبحث تبين أن:

المسرد (ة) نادية ربيع عبد القادر محمد بطاقة رقم 26703190102181
وطبقا الى خطاب من التأمينات الاجتماعية بغير عدم الانتفاع
بشهر من الفئات غير المنتفعة بالتأمين الصحي
وشكرا

تاريخ الطلب: 2023-1-23

الموظف المختص

منى محمد

قادة مركز نظم المعلومات
بمكتب

مسلسل 366174



الخطاب سارو لمدة ثلاثة اشهر من تاريخ الإصدار إلا أن حالة تغير حالة المواطن من قبل الهيئة العامة للتأمينات الاجتماعية أو
وزارة التأمين الاجتماعي أو وزارة التربية والتعليم

الإدارة المركزية لتكنولوجيا المعلومات ودعم القرار

NASSER INSTITUTE
ONCOLOGY CENTER
NUCLEAR MEDICINE UNIT



وزارة الصحة
إمارة إمارات الطبية المتخصصة
مستشفى ناصر
Nasser Institute Hospital
مستشفى ناصر
مركز الأورام
وحدة الطب النووي



Patient Name: Nadiah Rabie AbdElkader

Date: 17/11/2021

Tc-99m MDP bone scan.

Desired information : skeletal surveying to detect multiple lesions.

Technique:

Delayed Anterior and posterior whole body scan 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP.

Report:

Delayed Static Phase:-

- Degenerative changes are noticed over dorso-lumbar vertebrae.
- No definite hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are normally seen

Conclusion:

- The surveyed skeleton is free of osseous deposits.

Thanks for reference

Nuclear medicine Unit

Omnia Talaat, M.D

Patent No. 28763190102181

Sample: 19250/24

24. *Frangula*

Age: 53 y

Referring Doc.:

Request No. : 13004020

Request Date : 04/11/2024

Verification Date : 14/11/2024

Financial Category : اورام دار السلام - مبادرو

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Chief Complaint: Known case of breast cancer status post CBS presented with enlarged right breast, right neck and left axillary tail lesion.

Necktie or Specimen: Tra-cut biopsy, collected at 03/11/2024

Grass Experiments

1 Right axillary lymph node : Fragmented grayish white tissue cores up to 0.5 cm, totally

Fragmented grayish white tissue cores up to 0.5 cm, totally

Future Research Expectations

Immunohistochemical examination of both specimens labelled 1 and 2 reveals a cores of cores of intra-epithelial papillary structures with multiple hyperplastic lymphoid follicles with evidence of neoplasia or metastatic deposits in serial sections stained by IHC.

✓✓✓✓✓ Active staining in both sections:

History: 60-year-old female cancer status post CBS presented with enlarged right axillary lymph node. Pathologic examination of the right axillary lymph node: Tumor-free biopsy. No malignancy.

[illegible]

Signature

Dr. Rana Fathy

Dr. Nafiseh Moharezi

Salman Mohammad

Page 1 of 3

Printing By : Mrs. Mayada Hegazy

Patient Name: Nadia Rabeea Abdel Kader Mohamed	Date : 05/11/2024
Exam : C.T CHEST WITH CONTRAST	Age : 57 Year(s)
Medical ID: 2750/2021	Pat. ID: 10578

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to previous study dated 02.05.2023 the current study shows:

1. Chest:

- Regressive course regarding:
 - size of previously noted left breast axillary tail heterogenous spiculated mass now measuring 3.5x2.6 cm compared to 4.5x3.1 cm
 - size of left pathologically enlarged lymph nodes (level I, and level II), largest at level I now measuring 1x1 cm compared to 1.5x1 cm.
- Stationary course regarding:
 - Lingular segment post radiation changes.
 - Few bilateral scattered pulmonary nodules, the largest is noted at lateral segment of left lower lung lobe still measuring 6 mm.
- Bilateral few supraclavicular lymph nodes, largest on the right seen measuring 1.2 cm.
- Prominent right axillary lymph nodes, largest is seen measuring 1.5x1.1 cm (level I).
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- ***Almost Near total resolution*** of the previously noted bilateral anterior abdominal wall subcutaneous soft tissue densities/nodules.
- The liver is still seen mildly enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Small left renal cortical cyst noted.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Nadia Rabeea Abdel Kader Mohamed	Date : 05/11/2024
Exam : C.T CHEST WITH CONTRAST	Age : 57 Year(s)
Medical ID: 2750/2021	Pat. ID: 10578

- Small calcified anterior uterine wall fibroid.
- **Still noted rather unchanged** Previously noted left adnexal irregular hypodense lesion measures 4.5 x 4.2 cm ??.
- No abdominal or pelvic masses or collections.
- **The scanned skeleton shows**
 - L2 osteolytic lesion measuring about 1.5x1.1 cm
 - Left iliac bone osteosclerotic lesion measuring about 0.8 cm.
 - Right femoral neck bone island.

OPINION:

- Marked Regressive course regarding left breast mass, left axillary lymph nodes, and bilateral anterior abdominal densities.
- Right axillary lymph nodes and bilateral few supraclavicular lymph nodes for follow up.
- L2 osteolytic lesion, and left iliac bone osteosclerotic lesion for bone scan correlation.
- Stationary course regarding the rest of the study.

Dr. Basma Mohamed.

Dr. Walid Mohamed, M.D.

Patient Name : د. / نادية ربيع عبدالكادر
 Patient No. : 28703190102181
 Path No. : 1253/2022
 Sex : Female
 Age : 50 Y
 Referring Doc :
 Request No. : 13003111
 Request Date : 14/05/2022
 Verification Date : 22/05/2022
 Financial Category : أورام دار السلام - مبرارة
 Department : Out Patient
 Registered by : Ali mostafa

Pathology Report

Clinical Data: Lt breast cancer, Post neo-adjuvant chemotherapy

Nature of the specimen: Breast core biopsive surgery + ALND

Gross Examination:

- Procedure: A sector of breast tissue measured 5x4x2cm with skin ellipse measured 2.5x1cm
- Organization: 1
- Tumor extent
- Number: A firm greyish white mass
- Size: 2.5x1.5x1cm
- Skin findings: No grossly apparent
- Distance from margins: 1 cm from least inferior margin
- Lymph nodes number and size: 1 nodules up to 1cm
- Frozen section: - Free margins.

Microscopic examination:

- Histological type: Invasive Ductal carcinoma
- Grade (differentiation + nuclear pleomorphism + mitosis): GII (3+2+1)
- Angiolymphatic: No
- Ductal carcinoma in situ: > 25% comedo and non-comedo types
- Tumor extent: Actual tumour cellularity 85% = 2.1 cm
- Skin involvement: Free
- Lymph node involvement: Free (0) LNs
- Resection Margins:
- Free
- Histologic response to therapy: Pathological partial Response
- Additional pathologic findings: - Few lymphocytic infiltrate.

Verified by :
 dr.menna atef

Patient Name : الأستاذة / نادية ربيع محمد النور
Patient No. : 26703190102181
Path No. : 4250/24
Sex : Female
Age : 53 Y
Referring Doc :

Request No. : 13004020
Request Date : 04/11/2024
Verification Date : 14/11/2024
Financial Category : أورام دار السلام - مبنكوه
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Known case of breast cancer status post CBS presented with enlarged right axillary lymph nodes and left axillary tail lesion.
Nature of Specimen: Tru-cut biopsy, collected at 03/11/2024.

Gross Examination:

- 1) Right axillary lymph node : Fragmented grayish white tissue cores up to 0.5 cm, totally submitted.
- 2) Left axillary tail lesion: Fragmented grayish white tissue cores up to 0.5 cm, totally submitted.

Microscopic Examination:

- Histopathological examination of both specimens labelled 1 and 2 reveals a cores of cores of axial tissue showing patents sinuses with multiple hyperplastic lymphoid follicles with active germinal center. No evidence of neoplasia or metastatic deposits in serial sections examined confirmed by IHC

* IPT:

- CK7: Negative staining in both sections

Diagnosis:

Known case of breast cancer status post CBS presented with enlarged right axillary lymph nodes and left axillary tail lesion, Tru-cut biopsy:
Reactive nodal tissue . No malignancy.

Signature
Dr. Rana Pathy
Dr. Narmeen Mohammed

Verified by
DR Narmeen Mohammed

Printing Date : 28-11-2024 10:48:31

Page 1 of 2

Printing By : Mrs Mayada Hegazy